

PATIENT & SPECIMEN INFORMATION

|                     |                                   |           |                                |
|---------------------|-----------------------------------|-----------|--------------------------------|
| Patient name        | Hemingway, Ernest M.              | Accession | NGS-2026-052101                |
| Date of birth       | 1961-07-22 (Age 64)               | Specimen  | Oral swab, left lateral tongue |
| MRN                 | 10345621                          | Collected | 2026-05-19                     |
| Ordering physician  | Dr. Wisdom, DDS MD                | Received  | 2026-05-19                     |
| Clinical indication | Oral lesion — left lateral tongue | Reported  | 2026-05-20                     |

RESULT: VARIANTS DETECTED

DETECTED VARIANTS

| Gene   | HGVS (coding)         | Protein change | VAF   | Depth | Tier | Classification |
|--------|-----------------------|----------------|-------|-------|------|----------------|
| KRAS   | NM_004985.5:c.35G>T   | p.Gly12Asp     | 0.43% | 463×  | I    | Pathogenic     |
| PIK3CA | NM_006218.4:c.3140A>G | p.His1047Arg   | 0.70% | 714×  | I    | Pathogenic     |

VARIANT ANNOTATION

KRAS p.Gly12Asp | Pathogenic (Tier I)

KRAS p.Gly12Asp (G12D) is one of the most frequently detected oncogenic driver mutations in human cancer. It locks KRAS in the GTP-bound active state, constitutively activating downstream RAS/MAPK and PI3K/AKT/mTOR proliferative signalling. In oral squamous cell carcinoma, KRAS mutations are found in 3-8% of cases and mark a subgroup with aggressive behaviour. This variant is classified Pathogenic (Tier I, COSMIC ID COSM521). The co-occurrence with PIK3CA H1047R (see below) indicates dual activation of RAS and PI3K pathways, a combination associated with resistance to single-agent targeted therapies and may influence enrolment eligibility in clinical trials. NOTE: Observed VAF (0.43%) is above the 0.2% reporting threshold; this call is supported by 2 high-quality consensus reads at 463× consensus depth. The low consensus read count (VD = 2) reflects limited target coverage at 2,000× raw input; orthogonal confirmation is recommended before clinical action.

PIK3CA p.His1047Arg | Pathogenic (Tier I)

PIK3CA p.His1047Arg is a hotspot activating mutation in the kinase domain of the p110α catalytic subunit of PI3-kinase. It constitutively activates the PI3K/AKT/mTOR pathway, promoting cell survival, proliferation, and tumour angiogenesis. PIK3CA mutations occur in 15-20% of OSCC, most commonly at E545K and H1047R hotspots. This variant is classified Pathogenic (Tier I, COSMIC ID COSM775). PIK3CA H1047R is potentially actionable: alpelisib (PI3Kα inhibitor) is FDA-approved for PIK3CA-mutated breast cancer and is under investigation in HNSCC. Multidisciplinary tumour board review for consideration of PI3K-directed therapy or clinical trial enrolment is recommended.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EGFR, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, NFE2L2, NOTCH1, NSD1, PIK3R1, PTEN, TERT, TP53  
Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2×150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy, ≤1 edit distance), and molecular consensus sequences are called requiring ≥2 reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants ≥0.2% consensus VAF with ≥2 consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF ≥0.2% with validated sensitivity of 98.4% at ≥20,000× raw input depth (≥400× consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At 2,000× raw sequencing depth, approximately 15-20% of targets may fall below 100× consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

HOTSPOT SCREENING SUMMARY

**Patient:**

Hemingway, Ernest M.

**Accession:**

NGS-2026-052101

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFRCov ✓   |                |              | HRASCov ✓   |           |              |
|-------------|----------------|--------------|-------------|-----------|--------------|
| L858R       | c.2573T>G      | Not detected | G12V        | c.35G>T   | Not detected |
| T790M       | c.2369C>T      | Not detected | G12D        | c.35G>A   | Not detected |
| Exon 19 del | c.2235_2249del | Not detected | Q61L        | c.182A>T  | Not detected |
|             |                |              | Q61R        | c.182A>G  | Not detected |
| KRASCov ✓   |                |              | NOTCH1Cov ✓ |           |              |
| G12D        | c.35G>T        | ● DETECTED   | P2415L      | c.7244C>T | Not detected |
| G12V        | c.35G>A        | Not detected | R1699W      | c.5095C>T | Not detected |
| G12C        | c.34G>T        | Not detected |             |           |              |
| G13D        | c.38G>A        | Not detected | TP53Cov ✓   |           |              |
| PIK3CACov ✓ |                |              | R175H       | c.524G>A  | Not detected |
| E542K       | c.1624G>A      | Not detected | R248Q       | c.743G>A  | Not detected |
| E545K       | c.1633G>A      | Not detected | R248W       | c.742C>T  | Not detected |
| H1047R      | c.3140A>G      | ● DETECTED   | R273H       | c.818G>A  | Not detected |
| TERTCov ✓   |                |              | R282W       | c.844C>T  | Not detected |
| C228T       | c.-124C>T      | Not detected |             |           |              |
| C250T       | c.-146C>T      | Not detected |             |           |              |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

PATIENT & SPECIMEN INFORMATION

|                     |                                   |           |                                |
|---------------------|-----------------------------------|-----------|--------------------------------|
| Patient name        | Woolf, Virginia A.                | Accession | NGS-2026-052102                |
| Date of birth       | 1970-03-11 (Age 56)               | Specimen  | Oral swab, right buccal mucosa |
| MRN                 | 20456732                          | Collected | 2026-05-19                     |
| Ordering physician  | Dr. Wisdom, MD                    | Received  | 2026-05-19                     |
| Clinical indication | Oral lesion — right buccal mucosa | Reported  | 2026-05-20                     |

RESULT: VARIANTS DETECTED

DETECTED VARIANTS

| Gene   | HGVS (coding)         | Protein change | VAF   | Depth | Tier | Classification    |
|--------|-----------------------|----------------|-------|-------|------|-------------------|
| NOTCH1 | NM_017617.5:c.7244C>T | p.Pro2415Leu   | 0.26% | 769×  | II   | Likely Pathogenic |

VARIANT ANNOTATION

NOTCH1 p.Pro2415Leu | Likely Pathogenic (Tier II)

NOTCH1 functions as a tumour suppressor in stratified squamous epithelium. The p.Pro2415Leu missense variant affects a conserved residue in the PEST domain, predicted to disrupt protein stability (SIFT: deleterious; PolyPhen-2: probably damaging). NOTCH1 loss-of-function mutations are found in 10–15% of OSCC and are associated with early-onset tumourigenesis in the oral cavity. This variant is classified Likely Pathogenic (Tier II). In the context of confirmed dysplasia, detection of a NOTCH1 tumour suppressor variant supports malignant progression risk. Clinical correlation and multidisciplinary review are recommended. NOTE: Observed VAF (0.26%) is above the 0.2% reporting threshold and is supported by 2 consensus reads at 769× consensus depth. Orthogonal confirmation is recommended before clinical action.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EGFR, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, KRAS, NFE2L2, NSD1, PIK3CA, PIK3R1, PTEN, TERT, TP53

Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2×150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy, ≤1 edit distance), and molecular consensus sequences are called requiring ≥2 reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants ≥0.2% consensus VAF with ≥2 consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF ≥0.2% with validated sensitivity of 98.4% at ≥20,000× raw input depth (≥400× consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At 2,000× raw sequencing depth, approximately 15–20% of targets may fall below 100× consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

Reported by: Dr. Wisdom, MD PhD, FCAP

Report date: 2026-05-20

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

HOTSPOT SCREENING SUMMARY

**Patient:**Woolf, Virginia A.

**Accession:**NGS-2026-052102

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFRCov ✓   |                |              | HRASCov ✓   |           |              |
|-------------|----------------|--------------|-------------|-----------|--------------|
| L858R       | c.2573T>G      | Not detected | G12V        | c.35G>T   | Not detected |
| T790M       | c.2369C>T      | Not detected | G12D        | c.35G>A   | Not detected |
| Exon 19 del | c.2235_2249del | Not detected | Q61L        | c.182A>T  | Not detected |
|             |                |              | Q61R        | c.182A>G  | Not detected |
| KRASCov ✓   |                |              | NOTCH1Cov ✓ |           |              |
| G12D        | c.35G>T        | Not detected | P2415L      | c.7244C>T | ● DETECTED   |
| G12V        | c.35G>A        | Not detected | R1699W      | c.5095C>T | Not detected |
| G12C        | c.34G>T        | Not detected |             |           |              |
| G13D        | c.38G>A        | Not detected | TP53Cov ✓   |           |              |
| PIK3CACov ✓ |                |              | R175H       | c.524G>A  | Not detected |
| E542K       | c.1624G>A      | Not detected | R248Q       | c.743G>A  | Not detected |
| E545K       | c.1633G>A      | Not detected | R248W       | c.742C>T  | Not detected |
| H1047R      | c.3140A>G      | Not detected | R273H       | c.818G>A  | Not detected |
| TERTCov ✓   |                |              | R282W       | c.844C>T  | Not detected |
| C228T       | c.-124C>T      | Not detected |             |           |              |
| C250T       | c.-146C>T      | Not detected |             |           |              |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

**AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN**

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

PATIENT & SPECIMEN INFORMATION

|                     |                                   |           |                                |
|---------------------|-----------------------------------|-----------|--------------------------------|
| Patient name        | Dickens, Charles J.               | Accession | NGS-2026-052103                |
| Date of birth       | 1951-09-28 (Age 74)               | Specimen  | Oral swab, left floor of mouth |
| MRN                 | 30567843                          | Collected | 2026-05-19                     |
| Ordering physician  | Dr. Wisdom, DDS MD                | Received  | 2026-05-19                     |
| Clinical indication | Oral lesion — left floor of mouth | Reported  | 2026-05-20                     |

RESULT: VARIANTS DETECTED

DETECTED VARIANTS

| Gene | HGVS (coding)        | Protein change | VAF   | Depth | Tier | Classification |
|------|----------------------|----------------|-------|-------|------|----------------|
| TP53 | NM_000546.6:c.524G>A | p.Arg175His    | 0.60% | 664x  | I    | Pathogenic     |

VARIANT ANNOTATION

TP53 p.Arg175His | Pathogenic (Tier I)

TP53 p.Arg175His is the most frequently observed p53 mutation in human cancer. It disrupts the structural zinc-binding domain of the DNA-binding domain, abolishing wild-type tumour suppressor function, while also conferring dominant-negative and gain-of-function oncogenic properties including promotion of invasion, metastasis, and MDM2 antagonism (COSMIC ID COSM10648). In OSCC, TP53 mutations are present in 50–70% of cases and are associated with aggressive tumour behaviour, locoregional recurrence, and reduced sensitivity to platinum-based chemotherapy. The VAF of 0.60% in a surveillance swab warrants clinical correlation to exclude early local recurrence. Imaging and clinical re-evaluation are recommended.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EGFR, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, KRAS, NFE2L2, NOTCH1, NSD1, PIK3CA, PIK3R1, PTEN, TERT

Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2x150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy, ≤1 edit distance), and molecular consensus sequences are called requiring ≥2 reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants ≥0.2% consensus VAF with ≥2 consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF ≥0.2% with validated sensitivity of 98.4% at ≥20,000x raw input depth (≥400x consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At 2,000x raw sequencing depth, approximately 15–20% of targets may fall below 100x consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

Reported by: Dr. Wisdom, MD PhD, FCAP

Report date: 2026-05-20

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

HOTSPOT SCREENING SUMMARY

**Patient:**

Dickens, Charles J.

**Accession:**

NGS-2026-052103

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFR        |                |              | Cov ✓ |
|-------------|----------------|--------------|-------|
| L858R       | c.2573T>G      | Not detected |       |
| T790M       | c.2369C>T      | Not detected |       |
| Exon 19 del | c.2235_2249del | Not detected |       |
| KRAS        |                |              | Cov ✓ |
| G12D        | c.35G>T        | Not detected |       |
| G12V        | c.35G>A        | Not detected |       |
| G12C        | c.34G>T        | Not detected |       |
| G13D        | c.38G>A        | Not detected |       |
| PIK3CA      |                |              | Cov ✓ |
| E542K       | c.1624G>A      | Not detected |       |
| E545K       | c.1633G>A      | Not detected |       |
| H1047R      | c.3140A>G      | Not detected |       |
| TERT        |                |              | Cov ✓ |
| C228T       | c.-124C>T      | Not detected |       |
| C250T       | c.-146C>T      | Not detected |       |

| HRAS   |           |              | Cov ✓ |
|--------|-----------|--------------|-------|
| G12V   | c.35G>T   | Not detected |       |
| G12D   | c.35G>A   | Not detected |       |
| Q61L   | c.182A>T  | Not detected |       |
| Q61R   | c.182A>G  | Not detected |       |
| NOTCH1 |           |              | Cov ✓ |
| P2415L | c.7244C>T | Not detected |       |
| R1699W | c.5095C>T | Not detected |       |
| TP53   |           |              | Cov ✓ |
| R175H  | c.524G>A  | ● DETECTED   |       |
| R248Q  | c.743G>A  | Not detected |       |
| R248W  | c.742C>T  | Not detected |       |
| R273H  | c.818G>A  | Not detected |       |
| R282W  | c.844C>T  | Not detected |       |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

**AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN**

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

| PATIENT & SPECIMEN INFORMATION |                                                             |           |                            |
|--------------------------------|-------------------------------------------------------------|-----------|----------------------------|
| Patient name                   | Kafka, Franz J.                                             | Accession | NGS-2026-052105            |
| Date of birth                  | 1980-02-17 (Age 46)                                         | Specimen  | Oral swab, anterior tongue |
| MRN                            | 50789065                                                    | Collected | 2026-05-19                 |
| Ordering physician             | Dr. Wisdom, MD                                              | Received  | 2026-05-19                 |
| Clinical indication            | Oral lesion — anterior tongue (surveillance post-resection) |           | 2026-05-20                 |

RESULT: NO REPORTABLE VARIANTS DETECTED

INTERPRETATION

No pathogenic or likely pathogenic somatic variants were detected in the 21 genes covered by this assay at  $\geq 0.2\%$  VAF with  $\geq 2$  consensus supporting reads. A negative result does not exclude malignancy. Variants below the reporting threshold, variants outside the 251 assay target regions, structural variants, and copy number alterations are not assessed by this assay. Clinical correlation is required; repeat testing or biopsy should be considered if clinical suspicion persists.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EGFR, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, KRAS, NFE2L2, NOTCH1, NSD1, PIK3CA, PIK3R1, PTEN, TERT, TP53

Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2x150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy,  $\leq 1$  edit distance), and molecular consensus sequences are called requiring  $\geq 2$  reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants  $\geq 0.2\%$  consensus VAF with  $\geq 2$  consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF  $\geq 0.2\%$  with validated sensitivity of 98.4% at  $\geq 20,000\times$  raw input depth ( $\geq 400\times$  consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At  $2,000\times$  raw sequencing depth, approximately 15–20% of targets may fall below  $100\times$  consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

Reported by: Dr. Wisdom, MD PhD, FCAP

Report date: 2026-05-20

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

HOTSPOT SCREENING SUMMARY

**Patient:** Kafka, Franz J.

**Accession:** NGS-2026-052105

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFRCov ✓   |                |              | HRASCov ✓   |           |              |
|-------------|----------------|--------------|-------------|-----------|--------------|
| L858R       | c.2573T>G      | Not detected | G12V        | c.35G>T   | Not detected |
| T790M       | c.2369C>T      | Not detected | G12D        | c.35G>A   | Not detected |
| Exon 19 del | c.2235_2249del | Not detected | Q61L        | c.182A>T  | Not detected |
|             |                |              | Q61R        | c.182A>G  | Not detected |
| KRASCov ✓   |                |              | NOTCH1Cov ✓ |           |              |
| G12D        | c.35G>T        | Not detected | P2415L      | c.7244C>T | Not detected |
| G12V        | c.35G>A        | Not detected | R1699W      | c.5095C>T | Not detected |
| G12C        | c.34G>T        | Not detected |             |           |              |
| G13D        | c.38G>A        | Not detected | TP53Cov ✓   |           |              |
| PIK3CACov ✓ |                |              | R175H       | c.524G>A  | Not detected |
| E542K       | c.1624G>A      | Not detected | R248Q       | c.743G>A  | Not detected |
| E545K       | c.1633G>A      | Not detected | R248W       | c.742C>T  | Not detected |
| H1047R      | c.3140A>G      | Not detected | R273H       | c.818G>A  | Not detected |
| TERTCov ✓   |                |              | R282W       | c.844C>T  | Not detected |
| C228T       | c.-124C>T      | Not detected |             |           |              |
| C250T       | c.-146C>T      | Not detected |             |           |              |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

**AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN**

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.



| PATIENT & SPECIMEN INFORMATION |                                       |           |                                    |
|--------------------------------|---------------------------------------|-----------|------------------------------------|
| Patient name                   | Morrison, Toni E.                     | Accession | NGS-2026-052106                    |
| Date of birth                  | 1955-10-09 (Age 70)                   | Specimen  | Oral swab, left retromolar trigone |
| MRN                            | 60890176                              | Collected | 2026-05-19                         |
| Ordering physician             | Dr. Wisdom, MD                        | Received  | 2026-05-19                         |
| Clinical indication            | Oral lesion — left retromolar trigone | Reported  | 2026-05-20                         |

RESULT: NO REPORTABLE VARIANTS DETECTED

INTERPRETATION

No pathogenic or likely pathogenic somatic variants were detected in the 21 genes covered by this assay at  $\geq 0.2\%$  VAF with  $\geq 2$  consensus supporting reads. A negative result does not exclude malignancy. Variants below the reporting threshold, variants outside the 251 assay target regions, structural variants, and copy number alterations are not assessed by this assay. Clinical correlation is required; repeat testing or biopsy should be considered if clinical suspicion persists.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EGFR, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, KRAS, NFE2L2, NOTCH1, NSD1, PIK3CA, PIK3R1, PTEN, TERT, TP53

Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2x150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy,  $\leq 1$  edit distance), and molecular consensus sequences are called requiring  $\geq 2$  reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants  $\geq 0.2\%$  consensus VAF with  $\geq 2$  consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF  $\geq 0.2\%$  with validated sensitivity of 98.4% at  $\geq 20,000\times$  raw input depth ( $\geq 400\times$  consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At 2,000x raw sequencing depth, approximately 15–20% of targets may fall below 100x consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

Reported by: Dr. Wisdom, MD PhD, FCAP

Report date: 2026-05-20

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

HOTSPOT SCREENING SUMMARY

**Patient:**

Morrison, Toni E.

**Accession:**

NGS-2026-052106

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFRCov ✓   |                |              | HRASCov ✓   |           |              |
|-------------|----------------|--------------|-------------|-----------|--------------|
| L858R       | c.2573T>G      | Not detected | G12V        | c.35G>T   | Not detected |
| T790M       | c.2369C>T      | Not detected | G12D        | c.35G>A   | Not detected |
| Exon 19 del | c.2235_2249del | Not detected | Q61L        | c.182A>T  | Not detected |
|             |                |              | Q61R        | c.182A>G  | Not detected |
| KRASCov ✓   |                |              | NOTCH1Cov ✓ |           |              |
| G12D        | c.35G>T        | Not detected | P2415L      | c.7244C>T | Not detected |
| G12V        | c.35G>A        | Not detected | R1699W      | c.5095C>T | Not detected |
| G12C        | c.34G>T        | Not detected |             |           |              |
| G13D        | c.38G>A        | Not detected | TP53Cov ✓   |           |              |
| PIK3CACov ✓ |                |              | R175H       | c.524G>A  | Not detected |
| E542K       | c.1624G>A      | Not detected | R248Q       | c.743G>A  | Not detected |
| E545K       | c.1633G>A      | Not detected | R248W       | c.742C>T  | Not detected |
| H1047R      | c.3140A>G      | Not detected | R273H       | c.818G>A  | Not detected |
| TERTCov ✓   |                |              | R282W       | c.844C>T  | Not detected |
| C228T       | c.-124C>T      | Not detected |             |           |              |
| C250T       | c.-146C>T      | Not detected |             |           |              |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

**AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN**

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

PATIENT & SPECIMEN INFORMATION

|                     |                           |           |                        |
|---------------------|---------------------------|-----------|------------------------|
| Patient name        | Austen, Jane C.           | Accession | NGS-2026-052104        |
| Date of birth       | 1967-12-04 (Age 58)       | Specimen  | Oral swab, soft palate |
| MRN                 | 40678954                  | Collected | 2026-05-19             |
| Ordering physician  | Dr. Wisdom, MD            | Received  | 2026-05-19             |
| Clinical indication | Oral lesion — soft palate | Reported  | 2026-05-20             |

RESULT: VARIANTS DETECTED

DETECTED VARIANTS

| Gene | HGVS (coding)         | Protein change | VAF   | Depth | Tier | Classification |
|------|-----------------------|----------------|-------|-------|------|----------------|
| EGFR | NM_005228.5:c.2573T>G | p.Leu858Arg    | 0.57% | 525×  | I    | Pathogenic     |
| TP53 | NM_000546.6:c.524G>A  | p.Arg175His    | 0.30% | 669×  | I    | Pathogenic     |

VARIANT ANNOTATION

EGFR p.Leu858Arg | Pathogenic (Tier I)

EGFR p.Leu858Arg (L858R) is a well-characterised activating mutation in exon 21 of the EGFR kinase domain. It disrupts the autoinhibitory conformation, leading to constitutive EGFR kinase activation and downstream RAS/MAPK and PI3K/AKT signalling. While canonical in non-small cell lung adenocarcinoma (where it predicts response to EGFR TKIs such as osimertinib), EGFR L858R has been reported in 1–3% of OSCC. Cetuximab (anti-EGFR monoclonal antibody) is FDA-approved in HNSCC and may have enhanced activity in the setting of EGFR activating mutations. Multidisciplinary tumour board review for consideration of EGFR-directed therapy or clinical trial enrolment is recommended. Co-occurring TP53 R175H (see below) may affect response to EGFR-directed therapy and warrants combined consideration.

TP53 p.Arg175His | Pathogenic (Tier I)

TP53 p.Arg175His — see interpretation above (Dickens, Charles J., NGS-2026-052103). Co-occurrence of EGFR L858R and TP53 R175H has been associated with resistance to EGFR-directed therapies in other tumour types; multidisciplinary tumour board review is recommended. NOTE: Observed VAF (0.30%) is above the 0.2% reporting threshold and is supported by 2 consensus reads at 669× consensus depth. Orthogonal confirmation is recommended before clinical action.

GENES WITH NO PATHOGENIC OR LIKELY PATHOGENIC VARIANTS DETECTED

AJUBA, CASP8, CDKN2A, CUL3, DPYD, EP300, FAT1, FBXW7, HRAS, KEAP1, KMT2D, KRAS, NFE2L2, NOTCH1, NSD1, PIK3CA, PIK3R1, PTEN, TERT

Variants of uncertain significance (Tier III) are not reported.

METHOD SUMMARY

Cell-free and cellular DNA extracted from oral swab is subjected to hybridization capture using the IDT xGen Duplex UMI panel targeting 251 genomic regions (~77 kbp) across 21 cancer-relevant genes. Libraries incorporate 9-bp Unique Molecular Identifiers (UMIs) in a 9M1S+T read structure on both R1 and R2. Libraries are sequenced on Illumina NextSeq (2×150 bp). After alignment to GRCh38 (canonical chromosomes), reads are grouped by UMI using fgbio (adjacency strategy, ≤1 edit distance), and molecular consensus sequences are called requiring ≥2 reads per UMI family. Somatic variants are called on the consensus BAM with VarDict at minimum allele frequency 0.001. Variants ≥0.2% consensus VAF with ≥2 consensus supporting reads are reported. UMI deduplication reduces sequencing noise by >800-fold. Run-level negative controls are included on every sequencing run.

LIMITATIONS

This assay detects somatic single nucleotide variants and small indels within 251 target regions (~77 kbp) at VAF ≥0.2% with validated sensitivity of 98.4% at ≥20,000× raw input depth (≥400× consensus depth after UMI deduplication). Structural variants, copy number alterations, gene fusions, large indels, intronic variants outside defined capture regions, and variants in regions with low hybridization capture efficiency are not reliably assessed. A negative result does not exclude malignancy. Tumour heterogeneity, low tumour cell content, and specimen quality may affect variant allele fraction. Variants below the 0.2% VAF reporting threshold may be present but are not reported. At 2,000× raw sequencing depth, approximately 15–20% of targets may fall below 100× consensus depth; a result of no variant detected at low-coverage targets should be interpreted with caution. Results should be interpreted in the context of clinical and pathological findings by a qualified clinician.

Reported by: Dr. Wisdom, MD PhD, FCAP

Report date: 2026-05-20

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.

HOTSPOT SCREENING SUMMARY

**Patient:**

Austen, Jane C.

**Accession:**

NGS-2026-052104

Hotspots listed below were explicitly interrogated in addition to all variants above 0.2% VAF with ≥2 consensus reads across all 251 targets. Coverage criteria (≥100× consensus depth) were met at all targets.

| EGFR        |                |              | Cov ✓ |
|-------------|----------------|--------------|-------|
| L858R       | c.2573T>G      | ● DETECTED   |       |
| T790M       | c.2369C>T      | Not detected |       |
| Exon 19 del | c.2235_2249del | Not detected |       |

| KRAS |         |              | Cov ✓ |
|------|---------|--------------|-------|
| G12D | c.35G>T | Not detected |       |
| G12V | c.35G>A | Not detected |       |
| G12C | c.34G>T | Not detected |       |
| G13D | c.38G>A | Not detected |       |

| PIK3CA |           |              | Cov ✓ |
|--------|-----------|--------------|-------|
| E542K  | c.1624G>A | Not detected |       |
| E545K  | c.1633G>A | Not detected |       |
| H1047R | c.3140A>G | Not detected |       |

| TERT  |           |              | Cov ✓ |
|-------|-----------|--------------|-------|
| C228T | c.-124C>T | Not detected |       |
| C250T | c.-146C>T | Not detected |       |

| HRAS |          |              | Cov ✓ |
|------|----------|--------------|-------|
| G12V | c.35G>T  | Not detected |       |
| G12D | c.35G>A  | Not detected |       |
| Q61L | c.182A>T | Not detected |       |
| Q61R | c.182A>G | Not detected |       |

| NOTCH1 |           |              | Cov ✓ |
|--------|-----------|--------------|-------|
| P2415L | c.7244C>T | Not detected |       |
| R1699W | c.5095C>T | Not detected |       |

| TP53  |          |              | Cov ✓ |
|-------|----------|--------------|-------|
| R175H | c.524G>A | ● DETECTED   |       |
| R248Q | c.743G>A | Not detected |       |
| R248W | c.742C>T | Not detected |       |
| R273H | c.818G>A | Not detected |       |
| R282W | c.844C>T | Not detected |       |

REMAINING PANEL GENES — SEQUENCE-LEVEL DETECTION, NO PREDEFINED HOTSPOT LIST

**AJUBA · CASP8 · CDKN2A · CUL3 · DPYD · EP300 · FAT1 · FBXW7 · KEAP1 · KMT2D · NFE2L2 · NSD1 · PIK3R1 · PTEN**

All variants above 0.2% VAF with ≥2 consensus reads are reported for these genes. No variants meeting reporting criteria were detected. All targets in these genes met minimum coverage criteria (≥100× consensus depth).

**Reported by: Dr. Wisdom, MD PhD, FCAP**

**Report date: 2026-05-20**

Results reviewed and approved by the Laboratory Director. This report is electronically signed and does not require a wet signature.